CLINICAL TEACHING NOTE | FAMILY MEDICINE

Gastritis and Dyspepsia

Assessment and management in primary care

1. Diagnostic approach

Step 1: Classify by cause and course

- Acute gastritis: sudden onset, often related to NSAIDs, alcohol, or acute physiological stress (severe illness, burns), or acute H. pylori infection.
- Chronic gastritis: H. pylori-associated (most common, usually antral-predominant), autoimmune (body-predominant, associated with pernicious anaemia), or chronic NSAID-related.
- Erosive versus non-erosive gastritis: erosive disease carries a bleeding risk from mucosal breaks; non-erosive disease is inflammation without erosion.
- Functional dyspepsia: dyspeptic symptoms without an identifiable structural cause on investigation.

Step 2: Key history

- Symptom pattern: epigastric pain or burning, bloating, early satiety, nausea, and relationship to meals.
- ALARM features that mandate endoscopy: Anaemia, Loss of weight, Anorexia, Recent onset with a progressive course, Melaena or haematemesis, and Swallowing difficulty (dysphagia).
- Drug history: NSAIDs or aspirin (including low-dose aspirin), corticosteroids, bisphosphonates, SSRIs (which increase GI bleed risk when combined with NSAIDs), and anticoagulants.
- Alcohol intake, smoking, and dietary triggers.
- H. pylori risk factors and a family history of gastric cancer or peptic ulcer disease.
- Autoimmune disease history (e.g. vitiligo, thyroid disease), which is associated with autoimmune gastritis.
- New dyspepsia after around age 55 is itself considered an alarm feature under many guidelines.

Step 3: Examination

- Abdominal exam for epigastric tenderness, guarding or rigidity (suggesting perforation), or a mass (suggesting malignancy).
- Signs of anaemia: pallor, koilonychia, glossitis — also consider B12 deficiency in autoimmune gastritis.
- Signs of GI bleeding, including melaena on rectal exam if indicated, and haemodynamic instability.
- Signs of chronic liver disease if the history suggests alcohol-related gastritis.
- Weight and BMI trend, and lymphadenopathy (a Virchow's node raises concern for gastric malignancy).

2. Distinguishing the common causes

	H. pylori gastritis	NSAID-induced	Autoimmune gastritis	Functional dyspepsia	Peptic ulcer / malignancy (red flag)
Typical pattern	Chronic epigastric discomfort, often antral	Epigastric pain, may be silent until bleeding occurs	Body-predominant, often few GI symptoms	Postprandial fullness, early satiety, or epigastric pain without a structural cause	Persistent or progressive pain with weight loss

	H. pylori gastritis	NSAID-induced	Autoimmune gastritis	Functional dyspepsia	Peptic ulcer / malignancy (red flag)
Key history	Often asymptomatic; family history; lower socioeconomic exposure	Regular NSAID or aspirin use, often in older adults	Associated autoimmune disease; family history	No alarm features; often stress-related	Alarm features present; new onset after age 55
Key investigation	Stool antigen, urea breath test, or biopsy/rapid urease test	Clinical diagnosis; endoscopy if bleeding or alarm features	Parietal cell and intrinsic factor antibodies, B12 level, gastrin	Normal endoscopy — a diagnosis of exclusion	Urgent endoscopy
Complication risk	Peptic ulcer, MALT lymphoma, gastric cancer risk	Erosion, ulcer, GI bleeding or perforation	Pernicious anaemia, iron deficiency, increased gastric cancer/carcinoid risk	No structural complication; affects quality of life	Bleeding, perforation, gastric cancer
Course	Chronic unless eradicated	Improves on stopping the drug plus PPI therapy	Chronic, progressive mucosal atrophy	Chronic, relapsing–remitting	Needs prompt work-up

3. Investigations

H. pylori-negative, alarm-feature-free dyspepsia does not need extensive work-up. A reasonable primary-care set:

- H. pylori testing: stool antigen test or urea breath test — stop PPI therapy at least 2 weeks and antibiotics at least 4 weeks beforehand for accuracy; serology is useful only to exclude infection, not to confirm active or cured disease.
- FBC, ferritin, and CRP/ESR.
- LFTs if the history suggests alcohol-related or biliary symptoms.
- B12 level and intrinsic factor/parietal cell antibodies if autoimmune gastritis is suspected.
- Upper GI endoscopy with biopsy for any alarm feature, for new dyspepsia after age 55 (per local guideline threshold), or if symptoms fail to respond to initial treatment.
- Consider coeliac disease testing if bowel symptoms or unexplained anaemia coexist.

Red flags: same-day or emergency referral

- Haematemesis or melaena, or other signs of GI bleeding such as tachycardia, hypotension, or pallor.
- Progressive dysphagia or odynophagia.
- Unintentional weight loss, persistent vomiting, or an epigastric mass.
- Iron-deficiency anaemia without another clear cause.
- New dyspepsia at age 55 or older (or a lower threshold per local guideline) that persists.
- Signs of perforation: sudden severe abdominal pain, rigidity, or peritonism.

4. Management

General measures for all causes

- Stop or minimise causative drugs (NSAIDs, aspirin) where possible; co-prescribe a PPI if ongoing NSAID or aspirin use cannot be avoided.
- Advise reduced alcohol intake and offer smoking cessation support.
- Dietary modification: smaller meals, avoidance of known trigger foods, and avoiding lying down soon after eating.
- Explain the diagnosis and reassure once investigations are reassuring — many patients are worried about cancer.

Pharmacological management

Line	Drug	Typical dose	Key cautions
First line	Proton pump inhibitor (e.g. omeprazole)	20–40 mg once daily for 4–8 weeks	Long-term use carries a risk of B12/magnesium deficiency, fracture, and enteric infection — review the ongoing need periodically.
First line	H. pylori eradication triple therapy (PPI + amoxicillin + clarithromycin, or metronidazole if penicillin-allergic)	Standard combination dosing for 7–14 days per local protocol	Check local resistance patterns and allergy status; confirm eradication with repeat testing at least 4 weeks after completion.
Second line	H2-receptor antagonist (e.g. famotidine)	Standard dose	Less effective than a PPI; an alternative if PPI is not tolerated.
Second line	Bismuth-based quadruple therapy	Per local protocol	Used for penicillin allergy or after eradication failure.

Autoimmune gastritis

- Monitor and replace B12 (usually by IM injection) and iron as needed, with annual review of FBC and B12/folate.
- Arrange endoscopic surveillance per local or specialist guidance given the increased gastric cancer and carcinoid risk.
- Refer to gastroenterology for ongoing management and surveillance planning.

Functional dyspepsia

- First confirm H. pylori is negative or has been treated; trial a PPI or a prokinetic if symptoms persist.
- Consider a low-dose tricyclic antidepressant if symptoms persist despite standard therapy, with gastroenterology input.
- Address psychosocial contributors such as stress and anxiety as part of management.

Peptic ulcer disease / suspected malignancy

- Refer for urgent endoscopy and manage per specialist findings.
- Give eradication therapy for H. pylori-associated ulcers and biopsy any gastric ulcer to exclude malignancy.
- Re-endoscope to confirm healing of gastric ulcers.

5. Follow-up

- Reassess symptoms 4–8 weeks after starting a PPI or completing eradication therapy.
- Confirm H. pylori eradication with a stool antigen test or urea breath test at least 4 weeks after completing therapy and 2 weeks off PPI.

- Review the need for long-term PPI therapy periodically and step down to the lowest effective dose or on-demand use where possible.
- Re-investigate with endoscopy if symptoms recur or alarm features develop despite treatment.

Key take-home points

- ALARM features and new dyspepsia after age 55 mandate endoscopy, not empirical treatment alone.
- Test for and treat H. pylori before assuming symptoms are functional.
- Co-prescribe a PPI when ongoing NSAID or aspirin use cannot be avoided.
- Confirm H. pylori eradication after treatment — do not assume success.
- Autoimmune gastritis needs B12 and anaemia monitoring plus a specialist discussion about surveillance.

References

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This summary is an educational aid compiled from standard family medicine and specialty references; it is not a substitute for local clinical guidelines or the primary literature.